PHARVARIS N.V.

(NASDAQ: PHVS)

OUTPERFORM

LEERINK PARTNERS

Biopharma / Rare Diseases

Reason for report:

FLASH NOTE

Follow up: Liver Enzyme Elevations Not a Cause for Concern

September 8, 2026

Key Stats	
S&P 500 Health Care Index:	**1,996.03**
Price:	**$39.27**
52 Week High:	$38.13
52 Week Low:	$20.65
Shares Outstanding (mil):	68.1
Market Capitalization (mil):	$2,674.3

- **Bottom Line: Questions are circling around the two liver enzyme elevations reported in today's call—both patients had confounding factors, and we don't believe these adverse events (AEs) will warrant liver monitoring.** PHVS reported 3 Gr3 AEs: 1 tooth infection and 2 "Level 2" liver enzyme elevations (3.6%, 2/55). Per management, these elevations were 'slightly above' 5xULN and asymptomatic with no elevated bilirubin. Both patients had confounding factors: one was on long term androgen therapy (continued therapy and completed the trial) while the other had fatty liver disease (discontinued trial). By definition, a Gr3 elevation is >5-20x ULN, and it appears these elevations are on the low end of that range. We are encouraged that bilirubin was not affected and that this has not been seen in any prior studies. Recall, TAK's (Not Rated) Takhzyro had three liver enzyme elevations >3xULN and one elevation >8xULN in its Ph3, and one patient discontinued due to elevated LFTs (AST 4.1x ULN). Taken together, we do not believe these elevations reported today will warrant liver monitoring on the label.
- **Details on the safety profile from the call:**
  - There were 3 Gr3 AEs: 1 tooth infection and 2 "Level 2" liver enzyme elevations. Per mgmt, these elevations were 'slightly above' 5xULN and asymptomatic with no elevated billirubin. Both patients had confounding factors: either long term androgen use or fatty liver disease. One patient discontinued treatment and the other continued treatment and completed the study. The discontinuation in the pbo arm was due to urticaria.
  - There was 1 treatment emergent Gr4 SAE: laryngeal dyskinesia in a patient with piror history of repeated laryngeal intubation which was determined to be unrelated to treatment.
  - There were no GI side effects, and GI emergent AEs were balanced between treatment and pbo. Most common AEs were upper respiratory tract infections, nasopharyngitis and headache.

Joseph P. Schwartz
(617) 918-4575
joe.schwartz@leerink.com

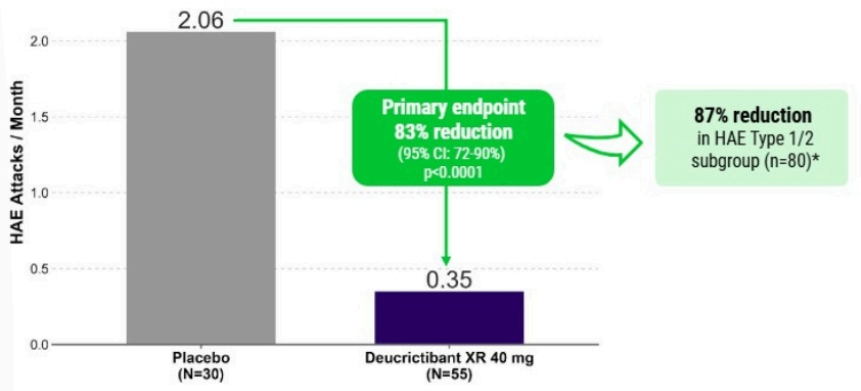
Leona Nease, Ph.D.
(212) 277-6095
leona.nease@leerink.com

Source: Company Information and Leerink Partners LLC Research.
General: Intra day price. Quarterly figures may not sum to annual total due to change in shares outstanding.

Please refer to page 3 for Important Disclosures, Price Charts and Analyst Certification.

- **Key points from the press release**
  - Treatment with deucrictibant XR in participants with all types of hereditary angioedema (HAE) reduced the mean monthly attack rate by 83% (p<0.0001).
  - Further analysis in the 80 participants with HAE Type 1 or Type 2 showed that the mean monthly attack rate was 87% lower in the deucrictibant XR group v. pbo.
  - All secondary efficacy endpoints, assessed sequentially under a multiplicity-control procedure, were also stat sig.
  - In CHAPTER-3, administration of deucrictibant XR resulted in early-onset protection within the first week, which was then sustained through the 24 weeks of study treatment.
  - **Safety:** In CHAPTER-3, deucrictibant XR was well tolerated with most treatment-emergent adverse events being mild or moderate. There were no treatment-related serious adverse events reported. One participant in each group discontinued treatment due to an adverse event.
  - **Next Steps:** CHAPTER-4 OLE study of deucrictibant XR for the prophylaxis of HAE attacks is ongoing. Topline data from Part 1 of CREAATE (NCT07266805), a global, pivotal Phase 3 study evaluating orally-administered deucrictibant XR for the prevention of AAE-C1INH attacks, are anticipated in the first quarter of 2027. Enrollment in CREAATE is ongoing and progressing as planned. PHVS plans to submit an NDA for prophylaxis deucrictibant in 1H27.

Exhibit 1: Deucrictibant XR reduced attack rate vs. pbo



*Additional analysis of HAE Type 1/2, not adjusted for multiplicity.
Notes: HAE Attacks / Month = time-normalized (per 4 weeks) number of investigator-confirmed HAE attacks during the 24-week treatment period; Results based on a Poisson regression model adjusted for treatment group, baseline attack rate, and age group.
Abbreviations: CI: confidence interval. HAE: hereditary angioedema. XR: extended-release.

Source: PHVS CHAPTER-3 press release

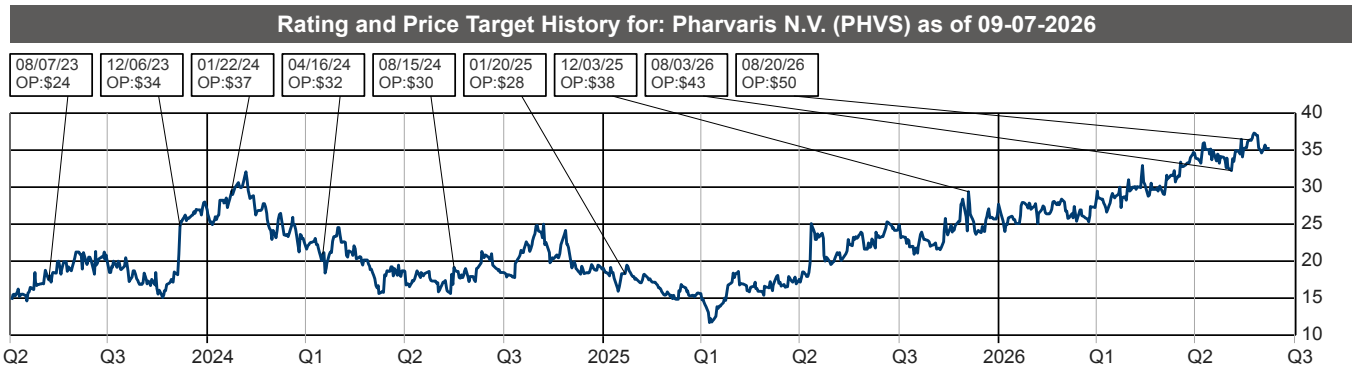
Disclosures Appendix

Completion: September 8, 2026 11:27 A.M. EDT.
Distribution: September 8, 2026 11:27 A.M. EDT.

Analyst Certification

I, Joseph P. Schwartz, certify that the views expressed in this report accurately reflect my views and that no part of my compensation was, is, or will be directly related to the specific recommendation or views contained in this report.

I, Leona Nease, Ph.D., certify that the views expressed in this report accurately reflect my views and that no part of my compensation was, is, or will be directly related to the specific recommendation or views contained in this report.



OP = Outperform MP = Market Perform UP = Underperform D = Drop Coverage I = Initiate SC = Suspended Coverage

Created by: BlueMatrix

Valuation

We derive a one-year PT of $50 for PHVS based on a DCF analysis using a 13% discount rate and a 2% terminal growth rate, converting the implied market cap from EUR to USD by using a 6-month FX average, representing an implied ~$3.4B market capitalization. Our valuation is driven by probability-adjusted revenue for the two different formulations of deucrictibant for HAE. We ascribe a 95% PoS for on-demand and a 60% PoS for prophylaxis.

Risks to Valuation

As a late-clinical-stage company, risks to PHVS include potential clinical, regulatory, and financial setbacks. Promising early clinical data may not translate to success in later-stage development. Regulatory decisions are unpredictable and may adversely affect the commercial potential of Pharvaris product candidates. Competition in the hereditary angioedema space (e.g., Takhzyro, Haegarda, Ekterly, Orladeyo) may limit commercial success(es) of Pharvaris products, if approved. From a financial perspective, dilutive financing is a potential risk. Other unanticipated situations could arise that may negatively or positively impact the stock.

Distribution of Ratings/Investment Banking Services (IB) as of 06/30/26				
			IB Serv./Past 12 Mos.	
Rating	Count	Percent	Count	Percent
BUY [OP]	241	80.3	142	58.9
HOLD [MP]	57	19.0	14	24.6
SELL [UP]	2	0.7	0	0

Explanation of Ratings

Outperform (Buy): We expect this stock to outperform its benchmark over the next 12 months.

Market Perform (Hold/Neutral): We expect this stock to perform in line with its benchmark over the next 12 months.

Underperform (Sell): We expect this stock to underperform its benchmark over the next 12 months.

The degree of outperformance or underperformance required to warrant an Outperform or an Underperform rating should be commensurate with the risk profile of the company.

For the purposes of these definitions the relevant benchmark for "Leerink Partners" branded healthcare and life sciences equity research will be the S&P 600® Health Care Index for issuers with a market capitalization of less than $2 billion and the S&P 500® Health Care Index for issuers with a market capitalization over $2 billion.

Important Disclosures

This information (including, but not limited to, prices, quotes and statistics) has been obtained from sources that we believe reliable, but we do not represent that it is accurate or complete and it should not be relied upon as such. All information is subject to change without notice. The information is intended for Institutional Use Only and is not an offer to sell or a solicitation to buy any product to which this information relates. Leerink Partners LLC (the "Firm" or "Leerink Partners"), its officers, directors, employees, proprietary accounts and affiliates may have a position, long or short, in the securities referred to in this report, and/or other related securities, and from time to time may increase or decrease the position or express a view that is contrary to that contained in this report. The Firm's research analysts, salespeople, traders and other professionals may provide oral or written market commentary or trading strategies that are contrary to opinions expressed in this report. The past performance of securities does not guarantee or predict future performance. Transaction strategies described herein may not be suitable for all investors. This document may not be reproduced or circulated without the Firm's written authority. Additional information is available upon request by contacting the Editorial Department, Leerink Partners, 53 State Street, 40th Floor, Boston, MA 02109.

Like all Firm employees, research analysts receive compensation that is impacted by, among other factors, overall firm profitability, which includes revenues from, among other business units, Institutional Equities, Research, and Investment Banking. Research analysts, however, are not compensated for a specific investment banking services transaction. To the extent Leerink Partners' research reports are referenced in this material, they are either attached hereto or information about these companies, including prices, rating, market making status, price charts, compensation disclosures, Analyst Certifications, etc. is available on https://leerink.bluematrix.com/sellside/Disclosures.action.

MEDACorp LLC, an affiliate of Leerink Partners, is a global network of independent healthcare professionals (Key Opinion Leaders and consultants) providing industry and market insights to Leerink Partners and its clients.

In the past 12 months, Leerink Partners has received compensation for providing investment banking services to Pharvaris N.V.

Leerink Partners expects to receive compensation for investment banking services from Pharvaris N.V. in the next 3 months.

Leerink Partners makes a market in Pharvaris N.V.

Leerink Partners has acted as a manager for a public offering of Pharvaris N.V. in the past 12 months.

This document may not be reproduced or circulated without our written authority. This document, and any other Leerink Partners research report may not be, in whole or in part, or in any form or manner (i) forwarded, distributed, shared, or made available to third parties, including as input to, or in connection with, any artificial intelligence or machine learning model; (ii) modified or otherwise used to create derivative works; or (iii) used to train or otherwise develop a generative artificial intelligence or machine learning model, without the express written consent of Leerink Partners. Receipt and review of this document constitutes your agreement with the aforementioned limitations in use.

© 2026 Leerink Partners LLC. All Rights Reserved. Member FINRA/SIPC. www.leerink.com

EQUITY RESEARCH TEAM

LEERINK PARTNERS

Research Management

Jim Kelly
Director of Equity Research
(212) 277-6096
jim.kelly@leerink.com

Christian Clark
Associate Director of Research
(212) 277-6117
christian.clark@leerink.com

Michelle Ko
Business Manager
(212) 277-6021
michelle.ko@leerink.com

Diversified Biopharmaceuticals

David Risinger, CFA
(212) 404-4539
david.risinger@leerink.com

Omari Baruti, Ph.D.
(212) 277-6218
omari.baruti@leerink.com

Jiayi Wang, Ph.D.
(212) 277-6152
jiayi.wang@leerink.com

Jason Zhuang
(212) 404-4552
jason.zhuang@leerink.com

Targeted Oncology

Andrew Berens, M.D.
(212) 277-6108
andrew.berens@leerink.com

Amanda Acosta-Ruiz, Ph.D.
(212) 404-4591
amanda.acostaruiz@leerink.com

Isabelle Lin
(212) 404-7228
isabelle.lin@leerink.com

Emily Shutman
(212) 404-4599
emily.shutman@leerink.com

Immuno-Oncology

Daina M. Graybosch, Ph.D.
(212) 277-6128
daina.graybosch@leerink.com

Jeffrey La Rosa
(212) 277-6103
jeffrey.larosa@leerink.com

Rabib S. Chaudhury, Ph.D.
(212) 277-6268
rabib.chaudhury@leerink.com

Bill Ling, Ph.D.
(212) 404-4550
bill.ling@leerink.com

Oncology

Asthika Goonewardene
(347) 226-0525
asthika.goonewardene@leerink.com

Carvey Leung
(212) 277-6068
carvey.leung@leerink.com

Karina Rabayeva
(212) 277-6147
karina.rabayeva@leerink.com

Genetic Medicine

Lili Nsongo, Ph.D.
(212) 277-6229
lili.nsongo@leerink.com

Immunology & Metabolism

Thomas J. Smith
(212) 277-6069
thomas.smith@leerink.com

Nat Charoensook, Ph.D., CFA
(212) 277-6264
nat.charoensook@leerink.com

Brian M. Conley, Ph.D.
(212) 277-6196
brian.conley@leerink.com

Emerging Immunology

Will Soghikian
(617) 918-4552
will.soghikian@leerink.com

Albert Agustinus, Ph.D.
(617) 918-4039
albert.agustinus@leerink.com

Neuroscience

Marc Goodman
(212) 277-6137
marc.goodman@leerink.com

Valentine Andreu, Ph.D.
(212) 277-6245
valentine.andreu@leerink.com

Alyssa Larios, Ph.D.
(212) 277-6093
alyssa.larios@leerink.com

Rare Disease

Joseph P. Schwartz
(617) 918-4575
joseph.schwartz@leerink.com

Jenny L. Gonzalez-Armenta, Ph.D.
(212) 277-6221
jenny.gonzalezarmenta@leerink.com

Leona Nease, Ph.D.
(212) 277-6095
leona.nease@leerink.com

Emerging Rare Disease

Basma Radwan, Ph.D.
(212) 277-6151
basma.radwan@leerink.com

Joseph P. Schwartz
(617) 918-4575
joseph.schwartz@leerink.com

Cardiovascular, Pulmonary & Endocrine Disorders

Roanna Ruiz, Ph.D.
(212) 277-6144
roanna.ruiz@leerink.com

Anna Li, Pharm.D.
(212) 277-6153
anna.li@leerink.com

Frank W. Zhong, Ph.D.
(212) 404-4534
frank.zhong@leerink.com

Life Science Tools & Diagnostics

Puneet Souda
(212) 277-6091
puneet.souda@leerink.com

Carlos Penikis, CFA
(212) 404-7225
carlos.penikis@leerink.com

Philip S. Song
(212) 404-4587
philip.song@leerink.com

Michael Sonntag
(212) 277-6048
michael.sonntag@leerink.com

Medical Devices and Technology

Mike Kratky, CFA
(212) 277-6111
mike.kratky@leerink.com

Samuil Gatev
(212) 277-6118
samuil.gatev@leerink.com

Healthcare Technology and Distribution

Michael Cherny
(212) 277-6189
michael.cherny@leerink.com

Daniel Clark
(212) 277-6233
daniel.clark@leerink.com

Leah D. Bailey
(212) 277-6106
leah.bailey@leerink.com

Dylan Finley, CFA
(212) 404-7231
dylan.finley@leerink.com

Ahmed Muhammad
(212) 404-4570
ahmed.muhammad@leerink.com

© 2026 Leerink Partners LLC. All Rights Reserved. Member FINRA/SIPC.

Healthcare Technology and Distribution / Animal Health

Daniel Clark
(212) 277-6233
daniel.clark@leerink.com

Michael Cherny
(212) 277-6189
michael.cherny@leerink.com

Healthcare Providers and Managed Care

Whit Mayo
(629) 802-2560
whit.mayo@leerink.com

Morgan T. McCarthy
(212) 277-6224
morgan.mccarthy@leerink.com

Dean Rosales
(212) 404-4590
dean.rosales@leerink.com

Editorial

SR. EDITOR/SUPERVISORY ANALYST

Thomas A. Marsilio
(212) 277-6040
thomas.marsilio@leerink.com

SUPERVISORY ANALYSTS

Robert Egan
bob.egan@leerink.com

Mike He
mike.he@leerink.com

Stuart MacDonnell
+44 7542 508454
stuart.macdonnell@leerink.com

Emily Singletary
(212) 277-6115
emily.singletary@leerink.com

Jose Yordan
(212) 404-7236
jose.yordan@leerink.com

© 2026 Leerink Partners LLC. All Rights Reserved. Member FINRA/SIPC.